

Chloroprocaine (systemic): Drug information - UpToDate

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For additional information see "Chloroprocaine (systemic): Patient drug information" and "Chloroprocaine (systemic): Pediatric drug information"

For abbreviations, symbols, and age group definitions show table

Brand Names: US

- Clorotekal [DSC];
- Nesacaine;
- Nesacaine-MPF

Brand Names: Canada

- Clorotekal

Pharmacologic Category

- Local Anesthetic

Dosing: Adult

Local anesthesia

Local anesthesia: Note: Use the smallest dose and concentration required to produce the desired result. Dosage varies with anesthetic procedure, the vascularity of the tissues, depth of anesthesia required, degree of muscle relaxation required, duration of anesthesia, and physical condition of the patient. Use reduced doses in debilitated patients and patients with cardiovascular disease. During epidural administration, a test dose (3 mL of 3% **or** 5 mL of 2%) is recommended prior to induction of complete block and all reinforcing doses with continuous catheter technique.

Clorotekal: Subarachnoid block (spinal anesthesia): **Intrathecal:** 1%: 50 mg single dose (effective block to the T 10 level). **Note:** Other preservative- and antioxidant-free formulations (eg, Nesacaine-MPF) have also been used off-label (dosage range: 20 to 60 mg) for spinal anesthesia for short-duration ambulatory procedures; ensure that preservative-free products do not contain the antioxidant sodium bisulfite prior to administration (Ref).

Nesacaine, Nesacaine-MPF:

Maximum single dose (without epinephrine): 11 mg/kg; maximum total dose: 800 mg.

Maximum single dose (with epinephrine 1:200,000): 14 mg/kg; maximum total dose: 1,000 mg.

Caudal block (preservative free): 2% or 3%: 15 to 25 mL; may repeat at 40- to 60-minute intervals.

Infiltration and peripheral nerve block:

Brachial plexus: 2%: 30 to 40 mL; total dose 600 to 800 mg.

Digital (without epinephrine): 1%: 3 to 4 mL; total dose: 30 to 40 mg.

Infraorbital: 2%: 0.5 to 1 mL; total dose 10 to 20 mg.

Mandibular: 2%: 2 to 3 mL; total dose 40 to 60 mg.

Paracervical: 1%: 3 mL per each of four sites; total dose: up to 120 mg.

Pudendal: 2%: 10 mL each side; total dose: 400 mg.

Lumbar epidural anesthesia, Cesarean delivery (preservative free): 3%: 15 to 20 mL total dose (includes test dose of 2 or 3 mL) (Ref).

Lumbar epidural anesthesia, nonpregnant (preservative free): 2% or 3%: 2 to 2.5 mL per segment; usual total volume: 15 to 25 mL; may repeat with doses that are 2 to 6 mL less than initial dose at 40- to 50-minute intervals.

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling. However, need for dosage adjustment unlikely due to limited duration of use.

Dosing: Liver Impairment: Adult

There are no specific dosage adjustments provided in the manufacturer's labeling; however, dosage should be reduced. Use with caution due to increased risk of adverse effects.

Dosing: Older Adult

Refer to adult dosing; in general, start at low end of dosage range.

Dosing: Pediatric

(For additional information see "Chloroprocaine (systemic): Pediatric drug information")

Dose varies with procedure, desired depth, and duration of anesthesia, desired muscle relaxation, vascularity of tissues, physical condition, and age of patient. The smallest dose and concentration required to produce the desired effect should be used. Decrease dose in debilitated patients and patients with cardiac disease.

Anesthesia, local injectable and peripheral nerve block

Anesthesia, local injectable and peripheral nerve block: Children >3 years and Adolescents: Maximum dose without epinephrine: 11 mg/kg; for infiltration, concentrations of 0.5% to 1% are recommended; for nerve block, concentrations of 1% to 1.5% are recommended

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Pediatric

There are no dosage adjustments provided in the manufacturer's labeling. Use with caution due to increased risk of adverse effects.

Dosing: Liver Impairment: Pediatric

There are no specific dosage adjustments provided in the manufacturer's labeling; however, dosage should be reduced. Use with caution due to increased risk of adverse effects.

Adverse Reactions

The following adverse drug reactions and incidences are derived from product labeling unless otherwise specified.

1% to 10%:

Cardiovascular: Hypotension (5%)

Endocrine & metabolic: Hyperglycemia (<2%)

Gastrointestinal: Nausea (<2%)

Local: Pain at injection site (4%)

Nervous system: Headache (<2%)

Postmarketing:

Cardiovascular: Bradycardia, cardiac arrhythmia, cardiac insufficiency, hypertension, presyncope, tachycardia

Dermatologic: Erythema multiforme

Gastrointestinal: Fecal incontinence, oral hypoesthesia, oral paresthesia, vomiting

Genitourinary: Groin pain, sexual disorder, urinary incontinence

Hypersensitivity: Hypersensitivity reaction (including anaphylaxis, angioedema, nonimmune anaphylaxis)

Nervous system: Akathisia, anxiety, arachnoiditis, burning sensation, cauda equina syndrome, dizziness, drowsiness, dysesthesia, feeling hot, hypoesthesia, loss of consciousness, malaise, motor dysfunction, myoclonus, paresthesia, peripheral neuropathy, restlessness, seizure, speech disturbance, tremor

Neuromuscular & skeletal: Back pain, limb pain

Ophthalmic: Blurred vision, diplopia, photophobia, visual disturbance

Otic: Auditory impairment, tinnitus

Respiratory: Apnea, dyspnea, respiratory depression

Contraindications

Hypersensitivity to chloroprocaine, other para aminobenzoic acid (PABA) ester type anesthetics, or any component of the formulation.

Additional contraindications described in the Clorotekal labeling (clinically, some may be applicable to other products): Contraindications specific to spinal anesthesia (eg, decompensated cardiac insufficiency, hypovolemic shock, coagulopathy); IV regional anesthesia; serious cardiac conduction problems; local infection at the site of proposed lumbar puncture; sepsis.

Canadian labeling: Additional Clorotekal contraindications (not in US labeling): Severe anemia; concurrent use with anticoagulants; acquired or congenital bleeding disorder.

Warnings/Precautions

Concerns related to adverse effects:

- Cardiovascular effects: Local anesthetics, at high systemic concentrations, are commonly associated with hypotension and bradycardia especially with inadvertent intravascular administration. Perform preventive measures (eg, limiting cumulative dose, use ultrasound or direct visualization for catheter placement). Careful and constant monitoring of the patient's cardiovascular vital signs should be done during and following each local anesthetic injection (Cox 2003; Dickerson 2014). In the event of cardiovascular collapse and/or severe CNS toxicity, treatment in accordance with the American Society of Regional Anesthesia and Pain Medicine's Checklist for Treatment of Local Anesthetic Toxicity is recommended (Neal [ASRA 2021]).

- CNS toxicity: Careful and constant monitoring of the patient's state of consciousness should be done following each local anesthetic injection; at such times, restlessness, anxiety, tinnitus, dizziness, blurred vision, tremors, depression, or drowsiness may be early warning signs of CNS toxicity. Use extreme caution in patients with existing neurological disease. Seizures due to systemic toxicity leading to cardiac arrest have also been reported, presumably following unintentional intravascular

injection. In the event of cardiovascular collapse and/or severe CNS toxicity, treatment in accordance with the American Society of Regional Anesthesia and Pain Medicine's Checklist for Treatment of Local Anesthetic Toxicity is recommended (Neal [ASRA 2021]).

- Intra-articular infusion related chondrolysis: Continuous intra-articular infusion of local anesthetics after arthroscopic or other surgical procedures is **not** an approved use; chondrolysis (primarily in the shoulder joint) has occurred following infusion, with some cases requiring arthroplasty or shoulder replacement.
- Methemoglobinemia: Has been reported with local anesthetics; clinically significant methemoglobinemia requires immediate treatment along with discontinuation of the anesthetic and other oxidizing agents. Onset may be immediate or delayed (hours) after anesthetic exposure. Patients with G6PD deficiency, congenital or idiopathic methemoglobinemia, cardiac or pulmonary compromise, exposure to oxidizing agents or their metabolites, or infants <6 months of age are more susceptible and should be closely monitored for signs and symptoms of methemoglobinemia (eg, cyanosis, headache, rapid pulse, shortness of breath, lightheadedness, fatigue).
- Respiratory effects: Local anesthetics have been associated with rare occurrences of sudden respiratory arrest. Careful and constant monitoring of the patient's respiratory (adequacy of ventilation) vital signs should be done following each local anesthetic injection.

Disease-related concerns:

- Cardiovascular disease: Use extreme caution in patients with cardiovascular disease, including severe hypertension, hypotension, heart block, and severe cardiac decompensation.
- Hepatic impairment: Use with caution in patients with severe hepatic impairment.
- Myasthenia gravis: Use with extreme caution in patients with myasthenia gravis; may cause significant weakness (Haroutiunian 2009).
- Plasma cholinesterase disorders: Use with caution in patients with genetic deficiency of plasma cholinesterase.
- Kidney impairment: Use with caution in patients with severe kidney impairment.

Special populations:

- Acutely ill patients: Use with caution in acutely ill; reduce dose consistent with age and physical status.
- Debilitated patients: Use with caution in debilitated patients; reduce dose consistent with age and physical status.
- Older adult: Use with caution in the elderly; reduce dose consistent with age and physical status.
- Pediatric: Use with caution in children; reduce dose consistent with age and physical status.

Other warnings/precautions:

- Administration: Intravascular injections should be avoided; aspiration should be performed prior to administration; the needle must be repositioned until no return of blood can be elicited by aspiration; however, absence of blood in the syringe does not guarantee that intravascular injection has been avoided.
- Epidural (thoracic, lumbar, or caudal) and intrathecal/spinal administration: **Do not use solutions containing preservatives.** Use extreme caution in patients with spinal deformities, neurologic disease, septicemia, and severe hypertension when used for thoracic, lumbar, and caudal epidural administration. With intrathecal/spinal administration, neurologic damage may occur after anesthesia (eg, paresthesia, loss of sensitivity, motor weakness, paralysis, cauda equina syndrome); symptoms may persist and may be permanent; carefully evaluate patients with underlying neuromuscular disorders while considering risk vs. benefit prior to treatment. Use with caution or avoid epidural or intrathecal/spinal administration in patients with bleeding disorders (congenital or acquired) and severe anemia (Horlocker 2010). Clorotekal (a product with specific FDA approval for intrathecal/spinal

administration) is contraindicated in patients with serious cardiac conduction abnormalities, local infection at proposed lumbar puncture site, or sepsis; contraindications to intrathecal/spinal anesthesia should be taken into consideration.

- Trained personnel: Clinicians using local anesthetic agents should be well trained in diagnosis and management of emergencies that may arise from the use of these agents. Resuscitative equipment, oxygen, and other resuscitative drugs should be available for immediate use.

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling. [DSC] = Discontinued product

Solution, Injection, as hydrochloride:

Nesacaine: 1% (30 mL); 2% (30 mL) [contains disodium edta, methylparaben (methyl p-hydroxybenzoate)]

Solution, Injection, as hydrochloride [preservative free]:

Nesacaine-MPF: 2% (20 mL); 3% (20 mL) [methylparaben free]

Generic: 2% (20 mL); 3% (20 mL)

Solution, Intrathecal, as hydrochloride [preservative free]:

Clorotekal: 1% (5 mL [DSC])

Generic Equivalent Available: US

Yes

Pricing: US

Solution (Chloroprocaine HCl (PF) Injection)

2% (per mL): \$1.28

3% (per mL): \$1.34

Solution (Nesacaine Injection)

1% (per mL): \$0.72

2% (per mL): \$0.74

Solution (Nesacaine-MPF Injection)

2% (per mL): \$1.66

3% (per mL): \$1.53

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Dosage Forms: Canada

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution, Intrathecal, as hydrochloride:

Clorotekal: 1% (5 mL)

Administration: Adult

Infiltration or peripheral nerve block: Administer locally as a single injection or continuously through an indwelling catheter (peripheral nerve block). Avoid rapid injection. Intravascular injections should be avoided; aspiration should be performed prior to administration; the needle must be repositioned until no return of blood can be elicited by aspiration; however, absence of blood in the syringe does not guarantee that intravascular injection has been avoided.

Epidural (thoracic, lumbar, or caudal) and subarachnoid block (spinal anesthesia): Do not use solutions containing preservatives. Use a filter needle to draw up solution from ampule when using Clorotekal. **Clorotekal is intended for intrathecal administration only;** the manufacturer recommends against using for epidural administration. Use of Clorotekal via continuous spinal catheters is not recommended (safety has not been established). Do not puncture areas of the skin with signs of infection/inflammation. Intravascular injections should be avoided; aspiration should be performed prior to administration; the needle must be repositioned until no return of blood can be elicited by aspiration; however, absence of blood in the syringe does not guarantee that intravascular injection has been avoided.

Administration: Pediatric

Parenteral: Administer locally as a single injection or continuously through an indwelling catheter. Avoid rapid injection. Do not use for subarachnoid administration. Intravascular injections should be avoided; aspiration should be performed prior to administration; the needle must be repositioned until no return of blood can be elicited by aspiration; however, absence of blood in the syringe does not guarantee that intravascular injection has been avoided.

Preparation for Administration: Pediatric

Parenteral: Nesacaine, Nesacaine-MPF: Dilute with NS. To prepare 1:200,000 epinephrine-chloroprocaine HCl injection, add 0.1 mL of a 1 mg/mL epinephrine injection to 20 mL of preservative-free chloroprocaine.

Storage/Stability

Store at 20°C to 25°C (68°F to 77°F) in original container; protect from freezing. Protect from light. Discard Clorotekal and Nesacaine-MPF following single use. Solution in vials may become discolored with prolonged exposure to light; do not administer discolored solutions. Crystals of chloroprocaine may develop when exposed to low temperatures; when the vial is returned to room temperature, the crystals will redissolve with shaking; do not use solutions that contain undissolved matter. Do not heat before use; do not autoclave. Use immediately after initial puncture of vial or after ampule opening.

Use: Labeled Indications

Local anesthesia:

Chloroprocaine (with preservatives):

Production of local anesthesia by infiltration and peripheral nerve block.

Chloroprocaine (without preservatives):

Clorotekal: Production of local anesthesia by subarachnoid block (spinal anesthesia) in adults.

Nesacaine-MPF: Production of local anesthesia by infiltration, peripheral, and central nerve block, including lumbar and caudal epidural block.

Note: Due to chloroprocaine's fast onset and short duration of action, it is most often used to establish adequate epidural anesthesia (eg, in a pregnant patient prior to delivery) or possibly, for peripheral nerve block in a patient undergoing short (<60 minutes) ambulatory surgery that is not anticipated to produce significant postoperative pain (Alley 2014; Gropper 2019).

Limitations of use: Nesacaine, Nesacaine-MPF: The manufacturer recommends against using either formulation (ie, with or without preservatives) for subarachnoid administration (ie, spinal anesthesia); however, the use of chloroprocaine without preservatives (Nesacaine-MPF) has been safely used off-label for spinal anesthesia (Gebhardt 2017; Gropper 2019; Wesselink 2022). Do not use chloroprocaine with preservatives (Nesacaine) for epidural or spinal anesthesia.

Medication Safety Issues

Sound-alike/look-alike issues:

Nesacaine may be confused with Neptazane

High alert medication:

The Institute for Safe Medication Practices (ISMP) includes this medication among its list of drug classes (epidural and intrathecal medications) which have a heightened risk of causing significant patient harm when used in error (ISMP List of High-Alert Medications in Acute Care Settings ©ISMP 2024).

Metabolism/Transport Effects

None known.

Drug Interactions

(For additional information: Launch drug interactions program)

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within “CYP3A4 Inducers [Strong]” are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the “Launch drug interactions program” link above.

Alpha-/Beta-Agonists: Chloroprocaine (Systemic) may increase hypertensive effects of Alpha-/Beta-Agonists. *Risk C: Monitor*

Anticoagulants: May increase adverse/toxic effects of Chloroprocaine (Systemic). *Risk C: Monitor*

Articaine: May increase adverse/toxic effects of Local Anesthetics. *Risk C: Monitor*

BUPIvacaine (Liposomal): Local Anesthetics may increase adverse/toxic effects of BUPIvacaine (Liposomal). Management: Liposomal bupivacaine should not be administered with local anesthetics, but may be administered 20 minutes or more after lidocaine. Avoid all local anesthetics within 96 hours after administration of liposomal bupivacaine. *Risk X: Avoid*

BUPIvacaine: Local Anesthetics may increase adverse/toxic effects of BUPIvacaine. Management: Avoid using any additional local anesthetics within 96 hours after insertion of the bupivacaine implant (Xaracoll) or bupivacaine and meloxicam periarticular solution (Zynrelef) or within 168 hours after subacromial infiltration (Posimir brand). *Risk C: Monitor*

Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates): Chloroprocaine (Systemic) may increase hypertensive effects of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk C: Monitor*

Hyaluronidase: May increase adverse/toxic effects of Local Anesthetics. *Risk C: Monitor*

Methemoglobinemia Associated Agents: May increase adverse/toxic effects of Local Anesthetics. Specifically, the risk for methemoglobinemia may be increased. *Risk C: Monitor*

Neuromuscular-Blocking Agents: Local Anesthetics may increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Phenylephrine (Systemic): Chloroprocaine (Systemic) may increase hypotensive effects of Phenylephrine (Systemic). *Risk C: Monitor*

PyRIDostigmine: May increase or decrease neuromuscular-blocking effects of Local Anesthetics. *Risk C: Monitor*

ROPivacaine: May increase adverse/toxic effects of Local Anesthetics. *Risk C: Monitor*

Sulfonamide Antibiotics: Chloroprocaine (Systemic) may decrease therapeutic effects of Sulfonamide Antibiotics. Management: Avoid concurrent use of chloroprocaine and systemic sulfonamide-based antimicrobials whenever possible. *Risk D: Consider Therapy Modification*

Technetium Tc 99m Tilmanocept: Coadministration of Local Anesthetics and Technetium Tc 99m Tilmanocept may alter diagnostic results. Management: Avoid mixing and simultaneously co-injecting technetium Tc 99m tilmanocept with local anesthetics. This interaction does not appear to apply to other uses of these agents in combination. *Risk C: Monitor*

Pregnancy Considerations

Chloroprocaine crosses the placenta (Kuhnert 1980).

Exposure to the fetus is expected to be limited due to the short maternal half-life but varies by route of administration (Nau 1985; Togioka 2022). Chloroprocaine may potentially cause varying degrees of maternal, fetal, and neonatal toxicity involving the CNS, peripheral vascular tone, and cardiac function depending on dose and administration technique.

Chloroprocaine is used in obstetric anesthesia (ACOG 2019; Novikova 2012; Reschke 2020). Use may be beneficial when a short onset of action is needed (Reschke 2020).

Breastfeeding Considerations

It is not known if chloroprocaine is present in breast milk.

According to the manufacturer, the decision to breastfeed during therapy should consider the risk of infant exposure, the benefits of breastfeeding to the infant, and the benefits of treatment to the mother. However, local anesthetics are considered compatible with breastfeeding (ABM [Reece-Stremtan 2017]; Mitchell 2020).

Monitoring Parameters

Cardiovascular and respiratory status; vital signs (eg, BP, heart rate, oxygen saturation); mental status changes; signs of CNS toxicity (eg, seizures, visual changes, muscle twitching).

Mechanism of Action

Chloroprocaine is an ester-type local anesthetic, which stabilizes the neuronal membranes and prevents initiation and transmission of nerve impulses thereby affecting local anesthetic actions. Chloroprocaine reversibly prevents generation and conduction of electrical impulses in neurons by decreasing the transient increase in permeability to sodium. The differential sensitivity generally depends on the size of the fiber; small fibers are more sensitive than larger fibers and require a longer period for recovery. Sensory pain fibers are usually blocked first, followed by fibers that transmit sensations of temperature, touch, and deep pressure. High concentrations block sympathetic somatic sensory and somatic motor fibers. The spread of anesthesia depends upon the distribution of the solution. This is primarily dependent on the volume of drug injected.

Pharmacokinetics (Adult Data Unless Noted)

Onset of action: 6 to 12 minutes

Duration (patient, type of block, concentration, and method of anesthesia dependent): Up to 60 minutes

Distribution: V_d : Depends upon route of administration; high concentrations found in highly perfused organs such as liver, lungs, heart, and brain

Metabolism: Rapidly hydrolyzed by plasma enzymes to 2-chloro-4-aminobenzoic acid and beta-diethylaminoethanol (80% conjugated before elimination)

Half-life elimination: In vitro, plasma: Neonates: 43 ± 2 seconds; Adults: 21 ± 2 seconds (males), 25 ± 1 second (females)

Excretion: Urine (minimal as unchanged drug in urine; metabolites: Chloro-aminobenzoic acid and beta-diethylaminoethanol primarily excreted unchanged)

Pharmacokinetics: Additional Considerations (Adult Data Unless Noted)

Hepatic function impairment: Pharmacokinetic parameters can be significantly altered.

Older adult: Pharmacokinetic parameters can be significantly altered.

Patient Counseling Points

(For additional information see "Chloroprocaine (systemic): Patient drug information")

What is this drug used for?

- It is used to numb an area before a procedure.

All drugs may cause side effects. However, many people have no side effects or only have minor side effects. Call your doctor or get medical help if any of these side effects or any other side effects bother you or do not go away:

- Irritation where the shot is given

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

For all uses of this drug:

- Too much acid in the blood (acidosis) like confusion; fast breathing; fast heartbeat; a heartbeat that does not feel normal; very bad stomach pain, upset stomach, or throwing up; feeling very sleepy; shortness of breath; or feeling very tired or weak
- Methemoglobinemia like a blue or gray color of the lips, nails, or skin; a heartbeat that does not feel normal; seizures; very bad dizziness or passing out; very bad headache; feeling very sleepy; feeling tired or weak; or shortness of breath
- Feeling lightheaded, sleepy, confused, or having blurred eyesight
- Sneezing
- Sweating a lot
- Fever
- Very upset stomach or throwing up
- Feeling nervous and excitable
- Restlessness
- Headache
- Anxiety
- Change in speech
- Numbness or tingling in the mouth
- Shakiness
- Dizziness or passing out
- Ringing in ears
- Low mood (depression)
- Change in balance
- Seizures

- Fast, slow, or abnormal heartbeat
- Chest pain or pressure
- Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Spinal:

- Trouble passing urine
- Loss of bladder or bowel control
- Not able to get or keep an erection
- Backache
- Long-lasting burning, numbness, tingling, or paralysis in the lower half of the body

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

Consumer Information Use and Disclaimer: This information should not be used to decide whether or not to take this medicine or any other medicine. Only the healthcare provider has the knowledge and training to decide which medicines are right for a specific patient. This information does not endorse any medicine as safe, effective, or approved for treating any patient or health condition. This is only a limited summary of general information about the medicine's uses from the patient education leaflet and is not intended to be comprehensive. This limited summary does NOT include all information available about the possible uses, directions, warnings, precautions, interactions, adverse effects, or risks that may apply to this medicine. This information is not intended to provide medical advice, diagnosis or treatment and does not replace information you receive from the healthcare provider. For a more detailed summary of information about the risks and benefits of using this medicine, please speak with your healthcare provider and review the entire patient education leaflet.

Brand Names: International

Find brand name(s) by country or region (for United States and Canada see separate Brand Names sections)

International Brand Names by Country

For country code abbreviations (show table)

- (AT) Austria: Ampres;
- (BE) Belgium: Ampres;
- (CH) Switzerland: Ampres | Chloroprocain HCl Sintetica | Ivracain;
- (CN) China: Ke pu nuo;
- (DE) Germany: Ampres;
- (ES) Spain: Ampres;
- (FI) Finland: Ampres;
- (FR) France: Clorotekal;
- (GB) United Kingdom: Ampres;
- (IT) Italy: Decelex;
- (LV) Latvia: Clorotekal;
- (NL) Netherlands: Ampres;
- (NO) Norway: Ampres;
- (PL) Poland: Ampres;
- (PR) Puerto Rico: Chloroprocaine HCL | Clorotekal;
- (SE) Sweden: Ampres;
- (SI) Slovenia: Decelex

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